TD SECURITIES (USA) LLC

September 8, 2026

Price: $9.96 (09/4/2026)
Price Target: $30.00

BUY (1)

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Key Data

Symbol	NASDAQ: BCRX
Market Cap	$2.5B

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QUICK TAKE: COMPANY UPDATE

Not A Zero Sum Game: Good PHVS Data, Controlled Orladeyo Pts Unlikely To Switch

THE TD COWEN INSIGHT

Oral competitor PHVS showed PhIII 83-87% attack rate reduction for deucritibant, which is good data. While this may impact Orladeyo's long-term growth, KOLs do not expect well-controlled Orladeyo pts to switch, as they already achieve injectable-like efficacy. BCRX's valuation already assumes significant competition, and we expect limited downside as we approach navenibart's PhIII in Q3:2027.

PHVS has reported Phase III data for oral deucritibant QD in HAE prophylaxis, with 83% attack-rate reduction in the overall population, and 87% attack-rate reduction in HAE Type 1 or 2 patients. This is competitive data, with injectable-like efficacy. Based on these results, PHVS expects to file for approval in H1:2027.

Although deucritibant is an improvement to Orladeyo's Phase III, investors still underappreciate the real-world clinical profile: ***the five-year retention rate for Orladeyo is impressive at ~50%, which sets a solid patient foundation*** - and *also suggests the other ~50% is low hanging fruit for PHVS to target.* This is not a zero-sum game, as consultants note well-controlled Orladeyo patients are achieving injectable-like efficacy, as mirrored by the KOL on the call: ***"there are patients who are doing extremely well on Orladeyo, and they would probably have little motivation to switch".*** This suggests PHVS is unlikely to meaningfully impact Orladeyo's already captured patient base - as there is no reason for Orladeyo patients to switch in H1:2028. While we expect long-term Orladeyo adoption may be impacted, BCRX's current valuation implies little downside (2x peak sales of ~$1B for combined Orladeyo/ navenibart).

We expect Orladeyo is more sticky than expected with PHVS' deucritibant launch, driving a durable and higher long-term revenue tail with IP through 2039 ($600-800MM+, vs. buy side expectations of $300-500MM+). Importantly, investors are positive on navenibart, with the Phase III data in Q3:2027 to establish BCRX as incumbent in HAE.

In a crowded prophylaxis HAE market, our KOL feedback suggests navenibart will lead within the injectable segment and Orladeyo will be the loyal option within the oral market. Our KOLs expect use for both oral and injectable prophylaxis, and a split in preference of 60-70% injectable/30-40% oral (here). We expect navenibart can capture ~20% of prophylaxis patients to achieve $1B in sales, even while sharing the HAE market. For Orladeyo, BCRX is guiding for peak sales of $800MM in the US, which implies 25-30% penetration of the 7,500 diagnosed/ treated patients, or ~2,000 patients on treatment for a full year of therapy.

On pipeline, BCX17725's (KLK5 inhibitor) POC PhI data in Netherton's syndrome are expected by YE. Navenibart's PhIII ALPHA-ORBIT completed enrollment in June 2026.

As for the Orladeyo launch, mgmt is executing on solid patient demand, with ***new prescriber additions (60+/mo) the same as 2025 - suggesting new injectable entrants are not impacting Orladeyo adoption***. For 2026, we expect Orladeyo sales of $640MM (+14%, vs. guidance of $625-645MM), with potential upside via peds oppy (~200 treated HAE patients, with prevalence of ~500 in the US). We view the ~25% penetration of peds Rx (August) as promising, and believe the peds launch should support Orladeyo growth in H2.

For the Orladeyo specifics, in Q2 we estimate there are ~1,490 paid patients in the U.S. For Orladeyo to reach $605MM in the US in 2026 would assume ~1,513 patients treated with paid drug for the full year, and assuming a retention rate of ~60-70% implies BioCryst will not require meaningful new patient additions.

HAE Prophylaxis Competitive Table For Approved Agents

Approved Prophylaxis Agents in HAE	Cinryze (pdC1INH)	Haegarda (C1INH)	Takhzyro (anti-pKal mAb)		Orladeyo (pKal inhibitor)	Andembry (FXIIa mAb)	Dawnzera (PKK ASO)		
	Takeda	CSL	Takeda		BioCryst	CSL	Ionis		
	Phase III	Phase III COMPACT	Phase III HELP		Phase III ApEX-2	Phase III VANGUARD	Phase II	Phase III OASIS-HAE	
Delivery	IV	SC	SC		Oral	SC	SC	SC	SC
Dose	BIW	BIW	300mg Q2W	300mg Q4W	150mg QD	400mg loading dose + 200mg Q4W	80mg Q4W	80mg Q4W	80mg Q8W
N	24	90	27	29	40	64	23	45	23
Baseline Monthly HAE Attack Rate	12.7 over 12 weeks on pcb	3.2	3.5	3.7	3.1	~3.0	~1.9	3.6	3.2
Mean HAE Attack Rate Reduction	51%	84%	87%	73%	44%	87%	90%	81%	55%
Data vs. Placebo/Baseline	Placebo	Placebo	Placebo		Placebo	Placebo	Placebo	Placebo	
Attack Free Rates at 3-Months	18%	40%	*N/A*		*N/A*	72%	92% (weeks 5-17)	*N/A*	
Placebo	*N/A*	0%	*N/A*		*N/A*	8%	*N/A*	*N/A*	
Attack Free Rates at 6-Months	*N/A*	*N/A*	44.4%	31%	*N/A*	62%	*N/A*	53%*	35%*
Placebo	*N/A*	*N/A*	2%		*N/A*	0%	*N/A*	9%	
Primary Endpoint	HAE attacks while on Cinryze vs. HAE attacks while on placebo	Time normalized number of HAE attacks	Monthly HAE attack rate reduction		Monthly HAE attack rate reduction	Monthly HAE attack rate reduction	Monthly HAE attack rate reduction	Monthly HAE attack rate reduction	
Efficacy On Primary Endpoint	6.3 over 12 weeks	Mean difference in attacks of 3.51 attacks per month for 60IU/kg doses	0.26	0.53	1.31	0.27	0.23	*N/A*	
Pbo/Baseline	12.7 over 12 weeks	*N/A*	1.97		2.35	2.01	2.21	*N/A*	
Proportion of patients achieving >90% Attack Rate Reduction			66.7%	55.2%	11-23% (150 mg dose, vs. 7.5% Pbo)	74% (vs. 8% Pbo)	92%		
Pbo			4.9%				0%		
Proportion of patients achieving >70% Attack Rate Reduction			88.9%	75.9%			92%		
Pbo			9.8%				0%		
Monthly Rate Of Attacks Requiring Acute Medication			87%	73%		88% lower than placebo	95% lower than placebo (weeks 5-17)		
Reduction In Monthly Rate Of Moderate To Severe Attacks Vs. Placebo/Baseline	*N/A*	*N/A*	74%	87%	*N/A*	90%	*N/A*	89%	41%
Timepoint	Week-24	Week-32	Week-24		Week-24	Week-24	Week-17	Week-24	

*IONS has reported attack free rates from Week 5 - 25

Source: TD Cowen, CSL, Takeda, BioCryst, Ionis Reports

HAE Prophylaxis Competitive Table For Developmental Stage Agents

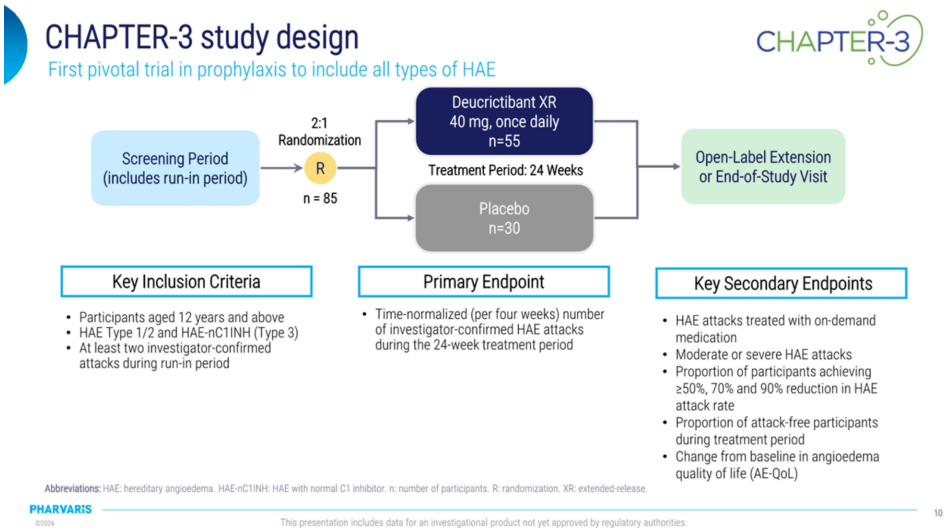
Developmental Prophylaxis Agents in HAE	Navenibart (anti-pKal mAb with YTE modification)			Deucrictibant (Bradykinin B2-RINH)			NTLA-2002 (KLKB1 Gene Knockout)					
	BioCryst Pharmaceuticals			Pharvaris			Intellia					
	Phase Ib/II ALPHA-STAR			Phase II HAE CHAPTER-1		Phase III CHAPTER-3	Phase I			Phase II		Phase III
Delivery	SC			Oral	Oral	Oral	IV			IV		IV
Dose	450mg	600mg loading dose + 300mg dose on Day 84	600mg loading dose + 600mg on Day 28	20mg QD	40mg QD	40mg QD	25mg	50mg	75mg	25mg	50mg	50mg
N	4	6	6	11	12	55	3	4	3	10	11	52
Baseline Monthly HAE Attack Rate	2.7	2.3	1.8	2.1	2.5	3.8	6	1.2	7.7	3.6	3.6	3.5
Mean HAE Attack Rate Reduction	90%, and 91% at 3-, and 6-months	95% at 3-, and 6-months	93% and 92% at 3-, and 6-months	79%	85%	83% overall; 87% in HAE Type 1/2	91%	97%	80%	75%	77%	87%
Data vs. Placebo/Baseline	Baseline			Placebo		Placebo	Baseline			Placebo		Placebo
Attack Free Rates at 3-Months	50%	67%	67%	40%	55%	*N/A*	67%	75%	33%	40%	73%	
Placebo	*N/A*			*N/A*		*N/A*	*N/A*			0%		
Attack Free Rates at 6-Months	25%	67%	67%	*N/A*		46%	*N/A*			*N/A*		62%
Placebo	*N/A*			*N/A*		3%	*N/A*			*N/A*		11%
Primary Endpoint	Monthly HAE attack rate reduction			Time normalized number of HAE attacks during treatment period			*N/A*			Monthly HAE attack rate reduction		
Efficacy On Primary Endpoint	0.27 and 0.23 at 3-, and 6-months	0.12 at 3-, and 6-months	0.12 and 0.15 at 3-, and 6-months	0.4	0.3	0.35	*N/A*			0.7	0.7	0.26
Pbo/Baseline	2.7	2.3	1.82	1.94		2.06	*N/A*			2.8		2.1
Proportion of patients achieving >90% Attack Rate Reduction						65.5%						
Pbo						10.0%						
Proportion of patients achieving >70% Attack Rate Reduction						96.4%						
Pbo						23.3%						
Monthly Rate Of Attacks Requiring Acute Medication												
Reduction In Monthly Rate Of Moderate To Severe Attacks Vs. Placebo/Baseline	95-96% at 6-months			83%	92%		95%			78%	85%	*N/A*
Timepoint	Weeks-12/24			Week-12		Week-24	Week-16			Week 5-28		

Source: TD Cowen, BioCryst, Pharvaris, Intellia Reports

Pharvaris Reports Phase III Results For Deucrictibant In HAE Prophylaxis

Pharvaris conducted a double-blind, placebo-controlled Phase III CHAPTER-3 study in HAE Type 1/2 and HAE Type 3 patients. The study enrolled N=85 patients, with 2:1 randomization across 40mg QD deucrictibant XR dosing arm and placebo. Primary endpoint of time-normalized monthly attack rate was measured following the 24 week treatment period, with patients later progressing to an OLE period.

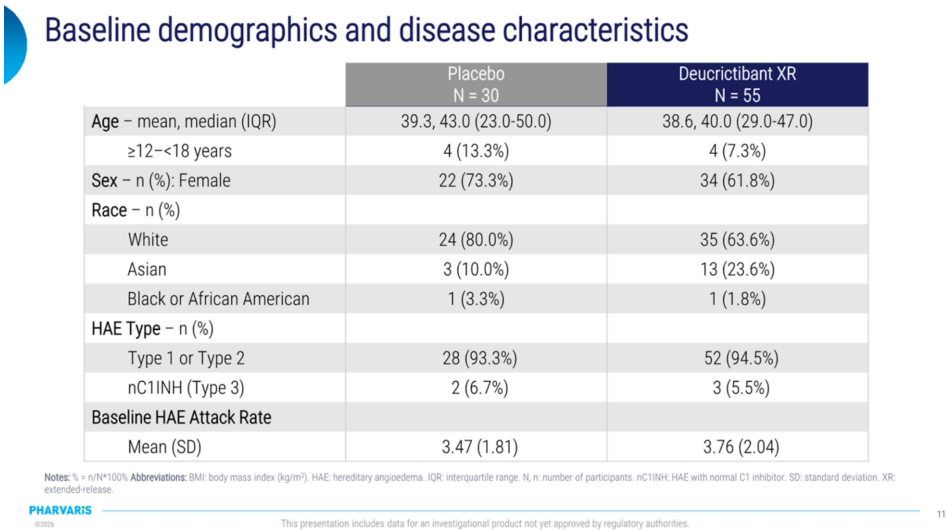
Deucrictibant Phase III CHAPTER-3 Study Design



Source: Pharvaris

Baseline characteristics were well-balanced across the two arms, with patients experiencing 3.47 mean monthly attack rate in the placebo group, and 3.76 in the 40mg QD deucrictibant group. This is in line with baseline disease burden across historical HAE prophylaxis trials.

Phase III CHAPTER-3 Baseline Demographics

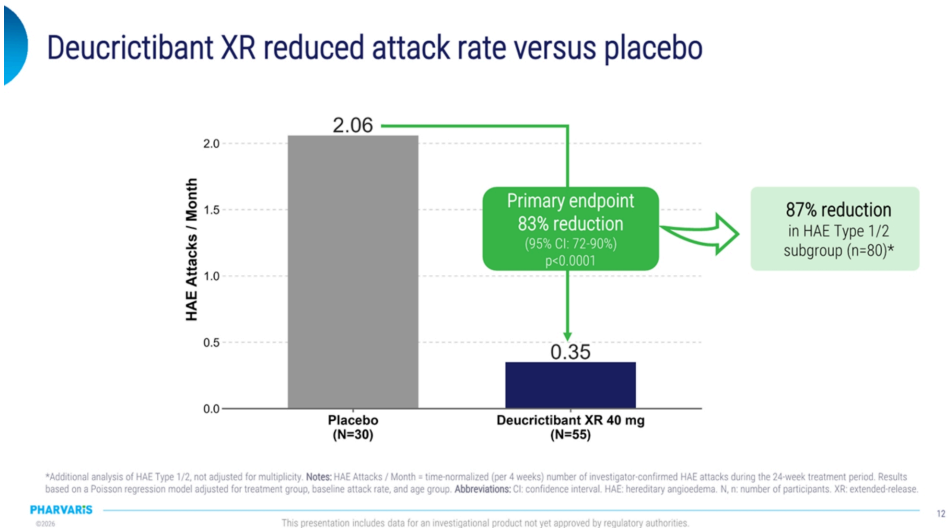


	Placebo N = 30	Deucrictibant XR N = 55
Age – mean, median (IQR)	39.3, 43.0 (23.0-50.0)	38.6, 40.0 (29.0-47.0)
≥12–<18 years	4 (13.3%)	4 (7.3%)
Sex – n (%): Female	22 (73.3%)	34 (61.8%)
Race – n (%)		
White	24 (80.0%)	35 (63.6%)
Asian	3 (10.0%)	13 (23.6%)
Black or African American	1 (3.3%)	1 (1.8%)
HAE Type – n (%)		
Type 1 or Type 2	28 (93.3%)	52 (94.5%)
nC1INH (Type 3)	2 (6.7%)	3 (5.5%)
Baseline HAE Attack Rate		
Mean (SD)	3.47 (1.81)	3.76 (2.04)

Source: Pharvaris

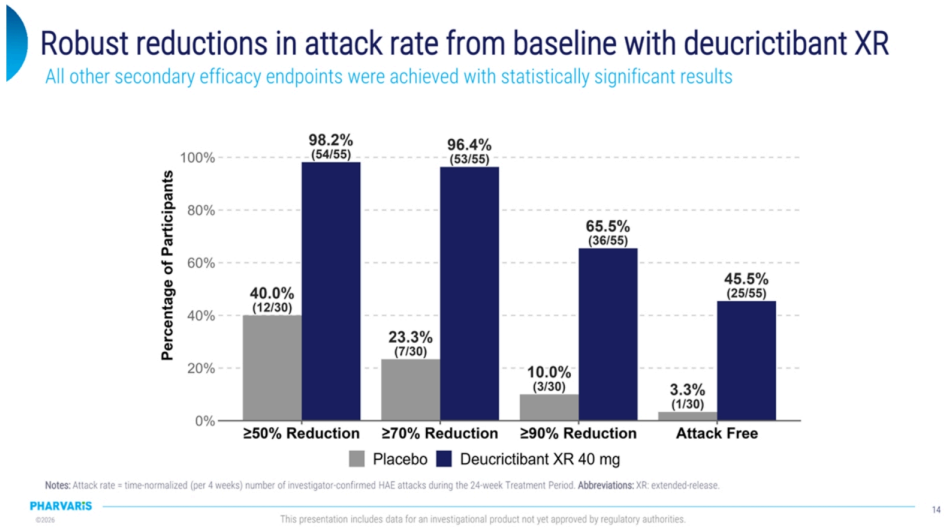
Deucrictibant XR achieved 83% attack rate reduction in the overall patient population, with 87% attack rate reduction in patients with Type1/2 HAE compared to placebo. In addition, deucrictibant XR achieved attack-free rate of 45.5% compared to 3.3% on placebo.

Phase III CHAPTER-3 Study Attack Rate Reduction



Source: Pharvaris

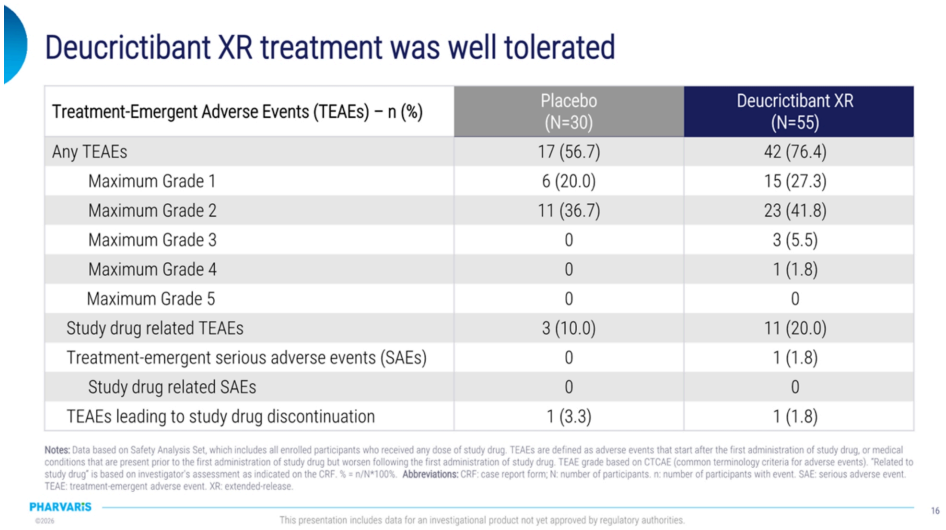
Phase III CHAPTER-3 Attack Free Rates



Source: Pharvaris

As for safety, Pharvaris reported one treatment-emergent serious adverse event (1.8%). Most common AEs were upper respiratory infection, nasopharyngitis, and headache. Three patients experienced Grade 3 TEAEs, including one patient with tooth infection, and two cases of liver enzyme elevation. One of the patients with liver enzyme elevation discontinued the study, while the other completed the study. One patient in the placebo arm discontinued the study due to urticaria.

Phase III CHAPTER-3 Safety/Tolerability



Deucrictibant XR treatment was well tolerated

Treatment-Emergent Adverse Events (TEAEs) – n (%)	Placebo (N=30)	Deucrictibant XR (N=55)
Any TEAEs	17 (56.7)	42 (76.4)
Maximum Grade 1	6 (20.0)	15 (27.3)
Maximum Grade 2	11 (36.7)	23 (41.8)
Maximum Grade 3	0	3 (5.5)
Maximum Grade 4	0	1 (1.8)
Maximum Grade 5	0	0
Study drug related TEAEs	3 (10.0)	11 (20.0)
Treatment-emergent serious adverse events (SAEs)	0	1 (1.8)
Study drug related SAEs	0	0
TEAEs leading to study drug discontinuation	1 (3.3)	1 (1.8)

Source: Pharvaris

We Believe Valuation Already Reflects Worst-Case Scenario Entry Of Competitive Oral Agent

Understandably, investors worry that additional prophylactic entrants could significantly impact Orladeyo share. However, our KOLs believe that well-controlled Orladeyo patients are unlikely to switch to new oral/injectable entrants, as they already experience injectable-like efficacy with their preference for an oral option. Our consultants expect a ~30-40%/60-70% split in orals/injectable use. We expect Orladeyo will have a significant first mover advantage - Pharvaris reported topline data from the Phase III study in September 2026 with study ongoing to capture one year safety data which suggests fastest approval/launch timelines of 2028.

Interestingly, on Pharvaris' Phase III data call the KOL reiterated that *"the responses are variable to Orladeyo"* but *"...there are patients doing extremely well on Orladeyo and I think they would probably have little motivation to switch".* This is a sentiment shared by our consultants, as they believe Orladeyo is performing better in the real world. We have high conviction that Pharvaris is unlikely to impact Orladeyo's already captured patient base. ***Stated more clearly, there would be no reason for Orladeyo patients (beginning in 2028/2029) to switch. That would mean deucrictibant would only impact new patients.*** Specifically, the long-term retention rate for Orladeyo is very stable at ~60%, which sets a solid patient foundation.

Orladeyo Launch Is Progressing Nicely

The company has indicated that the long-term patient retention at 12-months is roughly ~60%+, and is largely maintained beyond 12-months, with the falloff ~1-2% post one year. Encouragingly, the monthly discontinuation rate continued to decrease in 2025, as clinicians/patients gain comfort with the Orladeyo administration profile. In August 2026, BioCryst indicated steady quarterly patient adds, which suggests the new injectable product launches (CSL's Andembry and Ionis' Dawnzera) are not impacting the oral Orladeyo segment. BioCryst will switch to CareMed as the specialty pharmacy to fulfill shipment, which management hopes should help scale access/fulfillment. Pediatric Orladeyo Rx are already being filled via CareMed, with Rx for patients 12 years and older (Orladeyo capsules) to transition later in August. Management expects no long-term impact to Orladeyo growth/sales from the transition.

Regarding clinician engagement, management indicates strong addition of new prescribers, with increased penetration of the top 600 prescribers (that treat 50% of the U.S. HAE market).

As of May 2025, BioCryst reported that 81% of these top 600 prescribers have adopted Orladeyo and account for roughly half of new Orladeyo prescriptions. Management indicates that as of YE2025, 1,500+ US HCPs had prescribed Orladeyo, and 3,500+ patients have tried Orladeyo since launch. In August 2026, management highlighted that prescriber additions in Q2:2026 were above the previously indicated 60/month. BioCryst's market research suggests high clinician enthusiasm for Orladeyo, with 52% of allergists/immunologists indicating that they are “extremely likely to prescribe Orladeyo”.

Recall, ***management has previously discussed their long-term expectations for Orladeyo***, and believes that the consistent linear growth should likely reflect ~150 annual net patients added per year (adult and pediatric), off a base of ~1,600+ patients exiting 2025. We note for 2023 BioCryst reported 321 net total patient adds. For 2024, BioCryst indicated achieving similar total patients adds as in first year of launch, which is above their target of 200 net patient adds per year. As for the Orladeyo patient population, BioCryst has indicated that 52% of new Orladeyo patients are switching from other injectable prophylaxis therapies, 32% are switching from using only acute medications, and 17% are treatment naive.

To further inform best approaches to switching from other prophylactic therapies, BioCryst has initiated the Phase IV observational APeX-T. The trial will allow for both immediate switching, and a slower transition to Orladeyo with overlapping period of multiple prophylactic therapy use.

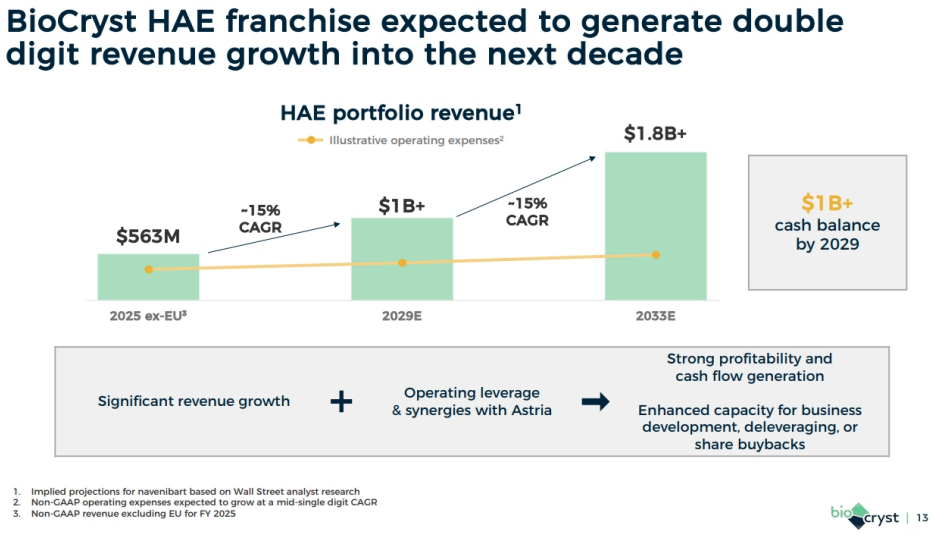
BioCryst has indicated that the paid rate was 84% in Q2:2026. Given the new patient adds, management had expected paid rate to decrease by 1-2% through the year, with further improvements expected during the reauthorization cycle in Q1/Q2. Management highlighted that ~67% of the improvement in paid rate in 2025 was due to the IRA redesign, with 89% of Medicare patients now on paid drug (vs. 55% in September 2024). Recall, with the 2025 IRA Medicare Part D redesign the out-of-pocket maximum is now reduced to $2,000, and patients are able to pay over the course of 12 months (vs. $3,250 out-of-pocket maximum due at once in 2024).

Long term, BioCryst expects improvements in the patient conversion from free to paid drug, which should also drive revenue growth as each 1% converted is ~$5MM in revenue. Currently, ~19% of patients are on free drug (including patients recently added to the quick start free paid program), which should be maintained or improved in the next few years. Within the commercial segment (~60% of Orladeyo patients), the company continues to see accelerated improvements in the transition to paid drug, with 84% of patients on paid drug as of May 2025.

For additional insight into Q2 patient numbers, we estimate that there are ~1,490 paid patients on Orladeyo in the U.S., or roughly +4% Q/Q patient growth from ~1,430 in Q1. We estimate ~ $149MM in US Orladeyo sales, assuming similar sales split as Q1 (94% US). Patient numbers are roughly calculated using Q2 U.S. sales of $149MM, divided by the net quarterly cost of ~$100K and assuming 84% of patients are receiving paid drug for a full quarter. This implies that ~1,773 patients are on treatment (vs. estimated 1,765 in Q1). For Q3, to achieve our U.S. Orladeyo sales expectations (~$160MM, +8% Q/Q), BioCryst would need to have ~1,600 patients on paid drug for the quarter.

Given the sales performance, we model $605MM in 2026 US Orladeyo sales, which implies ~1,513 patients treated for the full year (assuming net pricing of $400K). We believe this is achievable/could be exceeded given BioCryst reported 1,600 patients on Orladeyo exiting 2025, and guidance to add ~150+ annual net patients yearly. Correspondingly, we model $640MM (+14% Y/Y excluding EU sales) in global Orladeyo sales, at the higher end of the company's 2026 revenue guidance ($625-645MM, +13% Y/Y). Recall long term BioCryst has suggested “the same pattern based on patient growth trends and insurance seasonality.” For Orladeyo's quarterly cadence we expect steady growth through H2 (pediatric expansion).

As for the pediatric launch, management highlighted launch initiation on August 3rd with strong Rx demand (TRx 47 YTD). 50%+ of the Rx have completed prior authorization process, with high approval rates.



Source: BioCryst

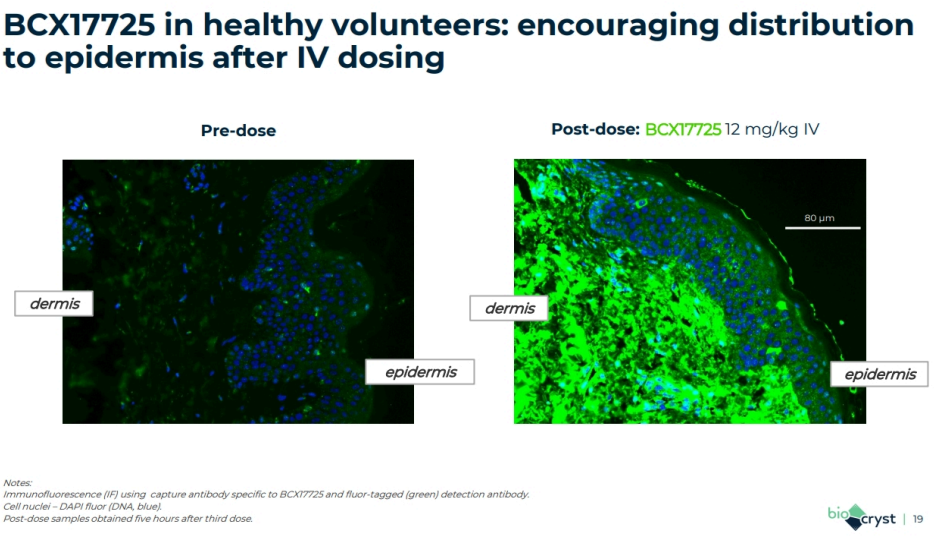
In October 2025, BioCryst completed the sale of its Orladeyo EU business to Neopharmed Gentili for $250MM in upfront cash and $14MM in milestone payments – with upfront cash value immediately accretive. Management expects the sale of Orladeyo EU business should reduce annual OpEx by over $50MM for breakeven vs. current EU sales and also improve Orladeyo margins. BioCryst used the cash proceeds to pay down $199MM of debt. In May 2026, BioCryst announced EU licensing agreement with Irish affiliate of Neopharmed Gentili for nevenibart in HAE. Per the licensing agreement, BioCryst received $70MM upfront payment, with up to $275MM in future regulatory and sales milestones, and 18-30% royalties on sales.

As for the Japanese launch, BioCryst indicated the current focus is to drive use of prophylaxis, and patient activation/education, especially as the oral prophylactic option. Regarding the opportunity, BioCryst highlighted that currently <1,000 HAE patients have been identified, out of expected 2,500-3,000 HAE patients. Of note, management indicated that Orladeyo price in Japan is ~$200k/year.

BCX17725's Early Data Is Promising

In November 2025, BioCryst presented encouraging early data from SAD/MAD studies of BCX17725 in healthy volunteers, showing localization of the drug in the dermis. Specifically, immunofluorescence stained images from tissue biopsies of healthy volunteers showed high concentration of BCX17725 in the dermis measured five hours post last dose (12 mg/kg IV) in the MAD study. In addition, BCX17725 was safe and well tolerated. BioCryst is now assessing BCX17725 in Netherton's patients with Part 3 (4 weeks of treatment) complete, and ongoing enrollment in Part 4 assessing long term dosing (12 weeks of treatment). The company expects to present a comprehensive dataset by YE2026.

BCX17725 Localizes In The Dermis Of Healthy Volunteers



Source: BioCryst Pharmaceuticals

Positive Interim Data From APeX-P Pediatric HAE Trial - August 2026 Launch

On December 12th, 2025 (PDUFA action date), BioCryst announced the FDA approval of Orladeyo (oral pellets) for HAE patients ages 2-12 years old, with launch initiated in August 2026 with the same prescriber targets as Orladeyo tablet (for adults). There is an unmet need for Orladeyo in this population, with a near-term opportunity in the ~500 diagnosed pediatric HAE patients. Notably, BioCryst has indicated that ~40% are treated, with the potential to capture both switch and new patients. In August, management highlighted 47 peds Rx (half completed PAs with high approval rate) with pellet shipment in later in the month. We also expect pediatric expansion will further drive overall patient demand for Orladeyo, to further entrench BioCryst as KOLs indicate well-controlled patients are unlikely to switch.

As for early metrics, BioCryst indicated 47 prescriptions YTD which is tracking ahead of the company's expectations for 2026. 50%+ of the Rx have completed prior authorization process with a high approval rate.

In February 2025, BioCryst reported positive interim results from the pediatric trial for Orladeyo in HAE (APeX-P). The APeX-P trial was designed to inform weight based dosing of granule formulation of Orladeyo for pediatric HAE patients. The trial includes an initial 12-week standard-of-care treatment period, followed by open-label Orladeyo treatment period lasting up to 144 weeks. The reported results include data from n=17 patients who had completed at least 48 weeks of Orladeyo treatment. A total of 29 patients have enrolled in the study.

Treatment with Orladeyo led to sustained monthly attack rate reduction. During the standard of care treatment period, the median monthly attack rate was 0.96, and mean monthly attack rate was 1.5. After one month of Orladeyo treatment, median monthly attack rate was reduced to 0, while mean monthly attack rate was reduced to 0.5. Through one year of treatment, median monthly attack rate was maintained, while mean monthly attack rate reduced to 0.3.

As for safety, Orladeyo showed a consistent AE profile to the adult and adolescent population and AEs were similar across all ages and weights. Tolerability was also favorable, with 25 patients (out of 29 total) having continued on Orladeyo.

In addition, BioCryst has also submitted regulatory filings in the EU and Japan, and plans to seek approval in additional geographies (i.e., Canada).

BioCryst Completes Acquisition Of Astria Therapeutics - Addition Of Navenibart Should Entrench BioCryst In HAE

In January 2025 BioCryst completed the ~$700MM EV acquisition of Astria Therapeutics' injectable HAE prophylaxis agent navenibart (funded by ~$400MM debt, 0.59 BCRX stock per ATXS share and cash). We believe navenibart could capture ~20% of HAE prophylaxis patients, with the potential to reach $1B in sales. This is a good addition, which should solidify BioCryst's position in HAE and support the Orladeyo momentum. Importantly, BioCryst expects to remain profitable and cash flow positive following the transaction.

Regarding navenibart development, KOLs expect high patient enthusiasm for Q3M dosing given the quality of life improvements, with Q6M dosing a game changer. As our KOL survey suggests, there is still a key role for Q3M given patient preferences. Navenibart has shown compelling POC, and we have high conviction for competitive efficacy in the Phase III ALPHA ORBIT study. The Phase II ALPHA-SOLAR OLE interim results (here) show an overall 92% mean attack rate reduction from baseline (avg 10.1 months of OLE follow-up), and 50% attack free rates (6 months follow-up) - and clean safety/ tolerability.

As for the HAE opportunity, our consultants expect use for both oral and injectable prophylaxis, and a split in preference of 60-70% injectable/30-40% oral (KOL survey here). We expect navenibart can capture ~20% of HAE prophylaxis patients to achieve $1B in sales, even while sharing the HAE market with other injectable/oral options.

We provide additional details on Astria's developmental progress for navenibart in our note here.

VALUATION METHODOLOGY AND RISKS

Valuation Methodology

Biotechnology:

In calculating our 12-month target price, we employ one or more valuation methodologies, which include a discounted earnings analysis, discounted cash flow analysis, net present value analysis and/or a comparable company analysis. These analyses may or may not require the use of objective measures such as price-to-earnings or price-to-sales multiples as well as subjective measures such as discount rates.

We make investment recommendations on certain early stage, pre-revenue companies based upon an assessment of their business model, technology, probability of market success, and the potential market opportunity, balanced by an assessment of applicable risks. Such companies may not be assigned a price target.

Investment Risks

Biotechnology:

There are multiple risks that are inherent with an investment in the biotechnology sector. Beyond systemic risk, there is also clinical, regulatory, and commercial risk. Additionally, biotechnology companies require significant amounts of capital in order to develop their clinical programs. The capital-raising environment is always changing and there is risk that necessary capital to complete development may not be readily available.

Risks To The Price Target

Risks to our price target include: (1) challenges to Orladeyo's IP from generics, (2) failure of BioCryst to develop navenibart, (3) unexpected competitive threats to the HAE market, (4) slower than anticipated launch and uptake of Orladeyo and/or navenibart, and (5) poor market penetration of Orladeyo and/or navenibart.

ADDENDUM

Stocks Mentioned In Important Disclosures

Ticker	Company Name
AVTX	Avalo Therapeutics
BCRX	BioCryst Pharmaceuticals
DMAC	DiaMedica Therapeutics
UCB	UCB SA

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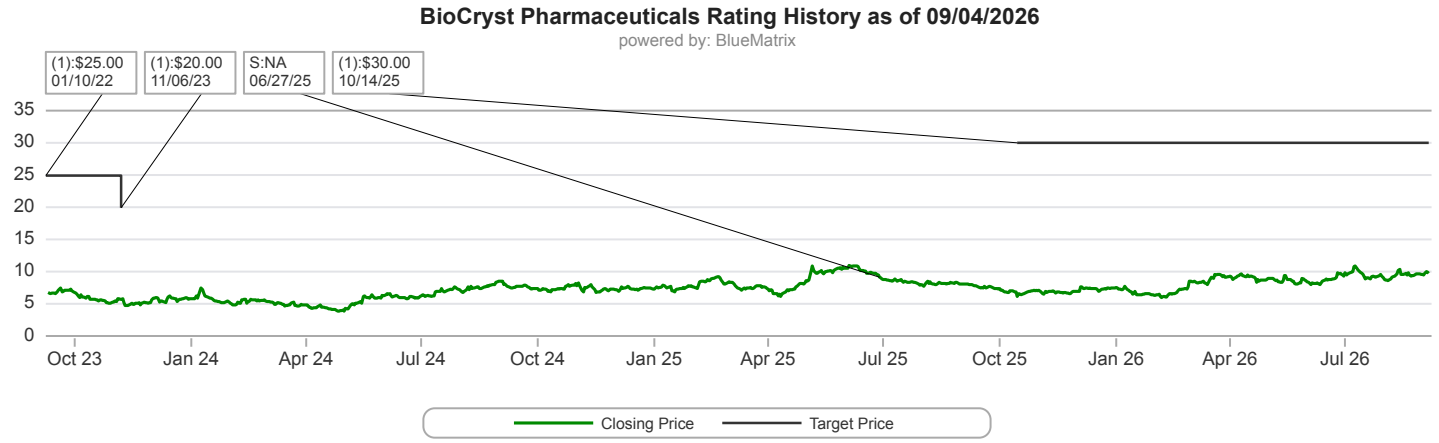
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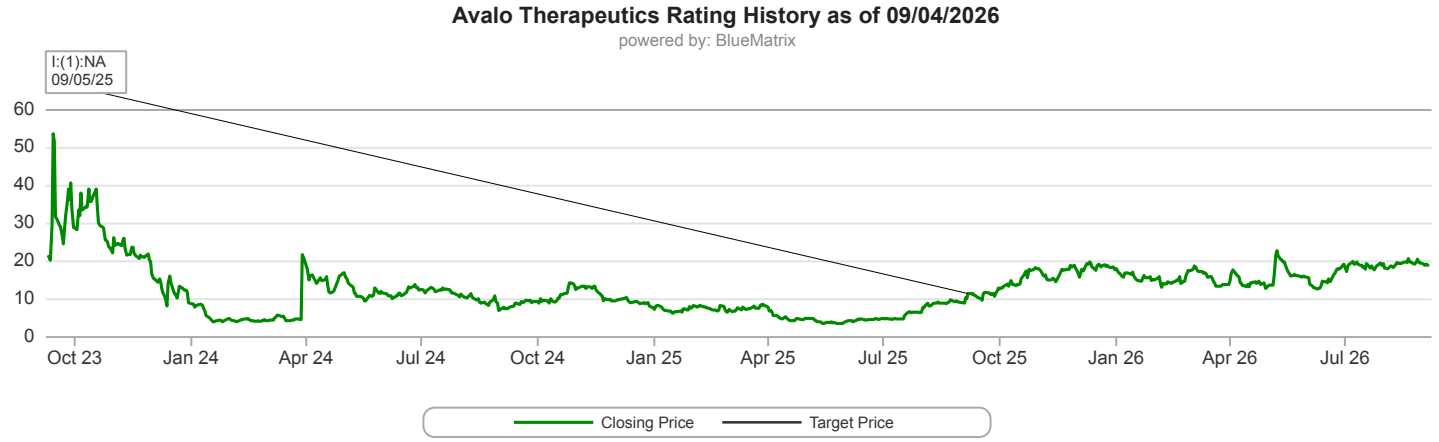
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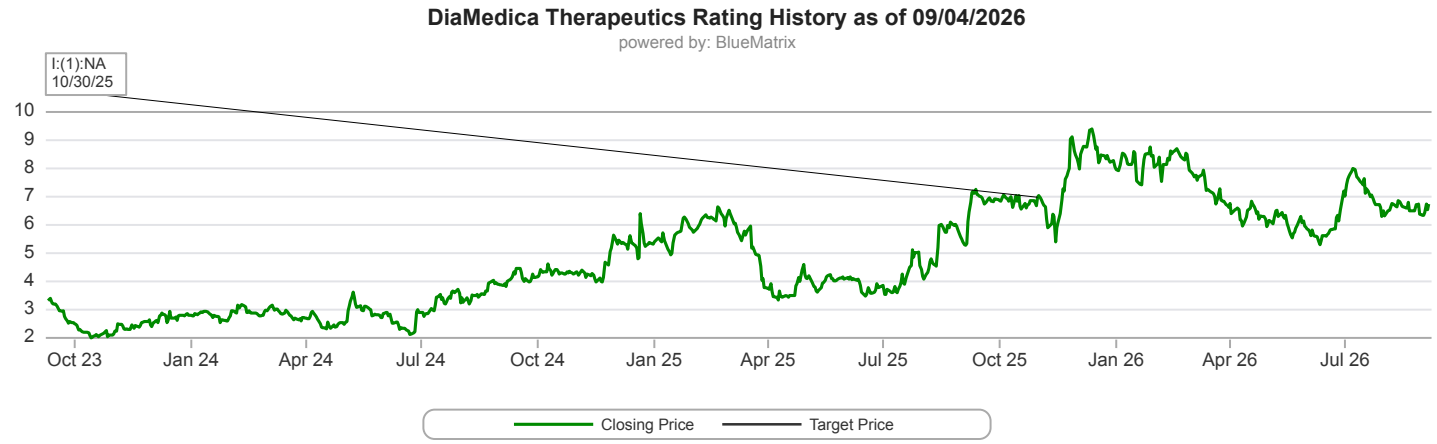
Distribution of Ratings/Investment Banking Services (IB) as of 06/30/26

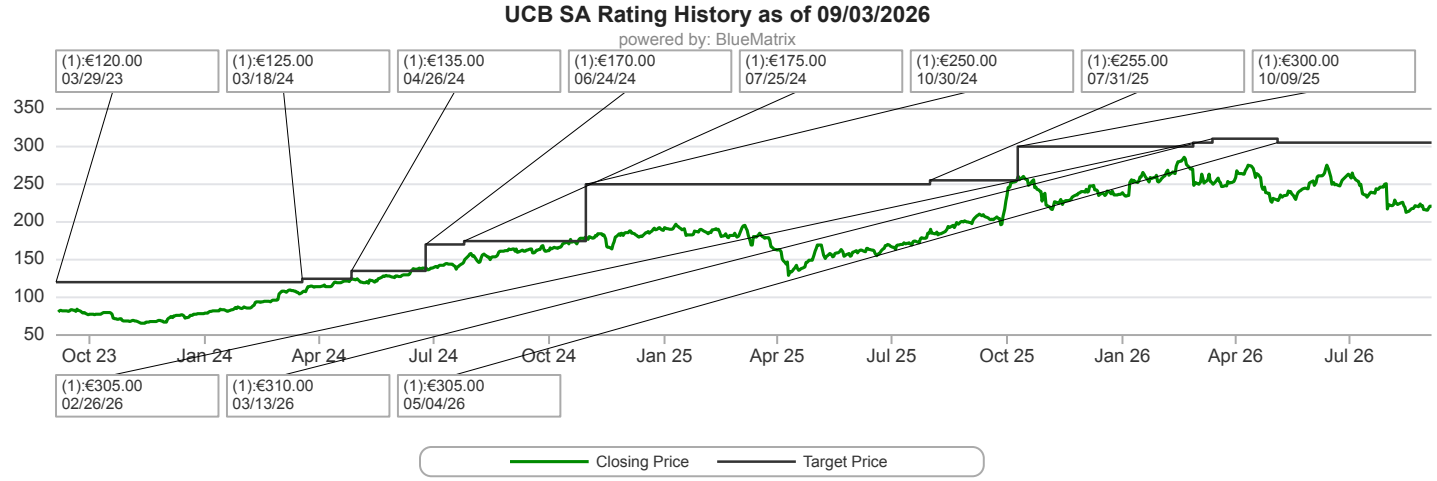
Rating	Count	Ratings Distribution	Count	IB Services/Past 12 Months
Buy	868	66.92%	348	40.09%
Hold	416	32.07%	130	31.25%
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